

Patient 11**Report A**

6521825 07/06/2016 CT Acute Stroke

Clinical History: Seizures, with history of hydrocephalus, hypertension, ministroke ?intracranial haemorrhage,

Findings:

Comparison is made with the previous CT dated 12/2/2016.

Streak artefacts are demonstrated, affecting visualisation of the posterior fossa.

There is a massive acute intraparenchymal haematoma involving most of the right frontal lobe, the right operculum and the right postcentral gyrus, with some extension to the right sylvian fissure, the lateral and third ventricles and minimally the left frontal region. There is a small focal haematoma also shown involving the left middle temporal gyrus. There is significant mass effect on underlying brain parenchyma including the right striatocapsular region, the brainstem and the lateral and third ventricles. Significant midline shift to the left, subfalcine herniation and descending transtentorial herniation are shown, with effacement of the basal cisterns. Areas of focal parenchymal hypoattenuation, most marked in the left inferior frontal and left postcentral region, likely related to changes of previous parenchymal haematomas. The possibility of amyloid angiopathy should be raised.

Urgent neurosurgical evaluation is advised.

Report B

6392607 10/02/2016 NHMR Head

Clinical details:

Normal pressure hydrocephalus. New right hand weakness. ? Stroke

Findings:

The previous MR brain dated 15/06/2015 has been reviewed (side-by-side comparison was not possible at the time of reporting). There are acute parenchymal haemorrhages centred on the subcortical white matter of the left postcentral gyrus, right middle frontal gyrus, and left orbitofrontal region. A modest amount of surrounding oedema is shown, with mild local mass effect. No midline shift or effacement of the basal cisterns is shown.

As previously demonstrated, extensive patchy T2-hyperintense signal change is shown within the cerebral white matter and pons, along with a cribriform appearance of the basal ganglia, in keeping with a background of severe small vessel disease. In addition, numerous peripheral parenchymal microhaemorrhages are again shown, with further areas of sulcal haemosiderin staining, which appear more extensive than previously.

Note is again made of prominence of the lateral ventricles slightly out of proportion to the degree of sulcal widening. A small meningioma is also again noted overlying the left medial occipital lobe.

Impression:

Acute parenchymal haemorrhages within both cerebral hemispheres as described, with mild associated local mass effect but no midline shift, and background changes suggestive of amyloid angiopathy.